

Acute headache Part 2

This pathway is intended as a continuation of the 'Acute headache Part I' ED proforma

DO NOT use this proforma on its own

Disclaimer: This is a clinical template; clinicians should always use judgment when managing individual patients

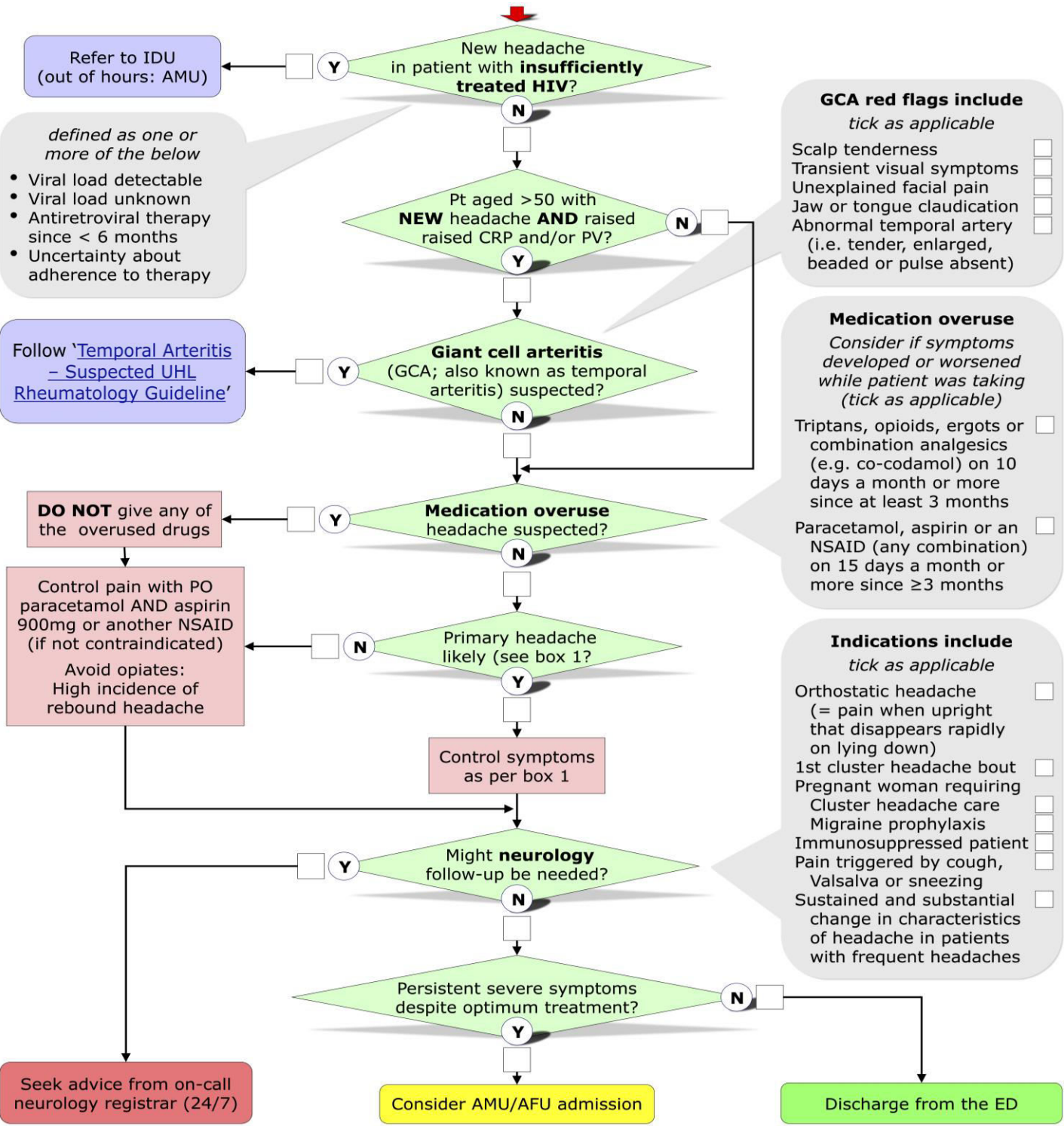
Patient details

Full name

DoB

Unit number

(use sticker if available)



Patient seen by

Print name

Signature

Position

Date

Time completed

① Primary headaches syndromes

consider the clinical features below and tick the most likely diagnosis (if applicable)

Primary headaches may be felt in head, face or neck. User the table below to choose the most effective treatment option, but be aware that a minimum number of characteristic episodes is needed to make a formal primary headache diagnosis (e.g. >9 for tension-type headache or >4 for migraine without aura. Medication overuse headache may make it difficult to diagnose any underlying primary headache.

To prescribe the appropriate analgesia in NC, go to Emergency Medicine (ED) > Analgesia (ED) > Primary headaches

Location of pain	Bilateral		Unilateral or bilateral			Unilateral (around or above the eye and along the side of the head / face)	
Quality	Non-pulsating (pressing / tightening)		Pulsating (throbbing or banging in young people aged 12-17 years)			Variable (can be sharp, boring, burning, throbbing or tightening)	
Intensity	Mild or moderate		Moderate or severe			Severe or very severe	
Duration	30 minutes – continuous		Adults: 4–72 hours Aged 12-17: 1–72 hours			15 – 180 minutes	
Effect on activities	Not aggravated by routine activities of daily living		Aggravated by, or causing avoidance of, routine activities of daily living			Restlessness or agitation	
Additional symptoms	None		Unusual sensitivity to light and / or sound, or nausea and / or vomiting Aura symptoms can occur with or without headache AND: • are fully reversible • develop over at least 5min • last 5 – 60min Typical aura symptoms include • visual symptoms such as flickering lights, spots or lines and / or partial loss of vision • sensory symptoms such as numbness and / or pins and needles • speech disturbance Atypical aura symptoms requiring referral to stroke team & neuroimaging (unless diagnosis has been firmly established previously) include • motor weakness • double vision • visual symptoms affecting only one eye • poor balance • decreased level of consciousness			On the same side as the headache: • red and / or watery eye • nasal congestion and / or runny nose • swollen eyelid • forehead and facial sweating • constricted pupil and / or drooping eyelid	
Frequency	<15 days per month	≥15 days per month for >3 months	≥15 days per month for >3 months	<15 days per month	predominantly between 2 days before and 3 days after the start of period in at least 2 out of 3 consecutive cycles	during a bout, 1 every other day up to 8 per day with remission between bouts >1 month	during a bout, 1 every other day up to 8 per day with a continuous remission between bouts <1 month in a 12-month period
Diagnosis tick as applicable	Episodic tension-type headache ☐	Chronic tension-type headache ☐ Overlap is common. If in doubt, treat as migraine.	Chronic migraine (+/- aura) ☐	Episodic migraine (+/- aura) ☐	Menstrual migraine (+/- aura) ☐	Episodic cluster headache ☐	Chronic cluster headache ☐
Control of acute symptoms	Paracetamol AND aspirin 900mg or another NSAID PO (if not contraindicated). <i>Avoid both in Parkinson's Disease and Lewy Body Dementia</i>		Prochlorperazine 12.5mg IM or metoclopramide IV/ IM (even if patient not feeling nauseated) AND paracetamol AND aspirin 900mg or another NSAID (if not contraindicated) AND sumatriptan 100mg PO (NB : if aged <18: zolmitriptan 5mg intranasally instead).		Same as for other types of migraine; GP may choose frovatriptan or zolmitriptan (both 2.5mg PO) if other triptans not effective.	Oxygen 12-15L/min via NRB mask AND zolmitriptan 5mg intranasally. GP may consider sumatriptan 6mg SC if the above is not effective.	
What to avoid	AVOID opioids – high risk of rebound headache.					DO NOT give opioids, paracetamol, aspirin or other NSAIDs, or oral triptans.	